National Cancer Control Programme (NCCP) 00614 · clinician draft

Carboplatin and vinorelbine · lung cancer

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Day 1 · carboplatin plus prescribed vinorelbine

Active medicines: Carboplatin + Vinorelbine

Emetogenic risk: high

Medicine	Dose / route	When
Aprepitant	125 mg orally Day 1; 80 mg orally Days 2–3	Before systemic anticancer therapy (SACT) on Day 1, then daily on Days 2–3
Ondansetron	16 mg orally	Day 1 Adjust for QT/individual risk.
Antiemetic dexamethasone reference	12 mg orally Day 1; 8 mg orally daily Days 2–4	Dedicated carboplatin AUC ≥4 NCIS V 8 row Reference course only: reconcile the aprepitant interaction and any other steroids into one clear prescription.
Metoclopramide	10 mg orally three times daily as prescribed	Subsequent days NCIS row does not specify an end day or PRN qualifier; prescriber/pharmacy must specify duration and suitability.

Additional phase-specific support

- Confirm prescribed vinorelbine route and review CYP3A4 inhibition by aprepitant; vinca toxicity may increase.
- Use one reconciled antiemetic prescription; do not add a second oral-vinorelbine5HT3agent to Day 1ondansetron.
- Use a constipation-prevention plan and assess bowel symptoms.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

National Cancer Control Programme (NCCP) 00614 v4 · pNCCP 00310 supportive care; NCIS V 8 Table 1 · NCCP NCIS antiemetics V 8

Sources checked 2026-11-02 · SACTCheck v0.84.29 · Regimen-level drafts; prescriber and oncology-pharmacy verification required

Day 8 · intravenous (IV) vinorelbine only

Active medicines: Vinorelbine

Emetogenic risk: Minimal IV risk

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- Individual antiemetic plan differs by route; list routine and rescue medicines separately.
- Discuss constipation prevention, hydration/diet where appropriate and prescribed laxatives; regular opioid use increases the need for a bowel plan.
- No routine steroid premedication, granulocyte colony-stimulating factor (white-cell growth support) (G-CSF), antibacterial, antiviral or Pneumocystis jirovecii pneumonia (PJP) prophylaxis is mandated by these National Cancer Control Programme (NCCP) monotherapy documents. Consider additional prevention based on individual risk/local policy; do not create a generic automatic prescription.
- Gastric protection is a consideration for high gastrointestinal-bleed risk, not a universal drug.
- Confirm strong CYP3A4 inhibitors/inducers, P-gp modulators, phenytoin, itraconazole, warfarin and live vaccines. Yellow fever vaccine is contraindicated. Avoid grapefruit per NCCP; do not self-start supplements.
- No routine prophylaxis required by NCCP minimal-risk pathway.
- Prescriber selects an as-needed medicine according to symptoms, constipation, QT/interactions and local policy. Reassess before future doses if nausea/vomiting occurs.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00614 v4 · p5 · NCCP antiemetic guidance v6 · minimal IV pathway

Sources checked 2026-11-02 · SACTCheck v0.84.29 · Regimen-level drafts; prescriber and oncology-pharmacy verification required

Day 8 · oral vinorelbine only if prescribed

Active medicines: Vinorelbine

Emetogenic risk: Moderate-to-high oral risk

Medicine	Dose / route	When
One 5-HT3 antagonist · choose a single option	Ondansetron 8–16 mg total daily OR granisetron 1–2 mg total daily	Around the weekly dose; the prescriber must state the number of subsequent days National Cancer Control Programme (NCCP) Table 5 oral pathway. Do not combine both agents or assume indefinite daily treatment. Review constipation, QT risk and interacting medicines.

Additional phase-specific support

- Individual antiemetic plan differs by route; list routine and rescue medicines separately.
- Discuss constipation prevention, hydration/diet where appropriate and prescribed laxatives; regular opioid use increases the need for a bowel plan.
- No routine steroid premedication, granulocyte colony-stimulating factor (white-cell growth support) (G-CSF), antibacterial, antiviral or Pneumocystis jirovecii pneumonia (PJP) prophylaxis is mandated by these NCCP monotherapy documents. Consider additional prevention based on individual risk/local policy; do not create a generic automatic prescription.
- Gastric protection is a consideration for high gastrointestinal-bleed risk, not a universal drug.
- Confirm strong CYP3A4 inhibitors/inducers, P-gp modulators, phenytoin, itraconazole, warfarin and live vaccines. Yellow fever vaccine is contraindicated. Avoid grapefruit per NCCP; do not self-start supplements.
- Table 5 says continue daily; for once-weekly vinorelbine the prescriber must state the number of days. Do not automatically turn this into indefinite daily treatment or import an intravenous (IV) moderate steroid/NK1 schedule.
- A separate locally approved as-needed plan; check constipation and QT/interactions. Avoid duplicate 5-HT3 agents. Ondansetron can worsen constipation.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00614 v4 · p5 · NCCP antiemetic guidance v6 · oral Table 5

Sources checked 2026-11-02 · SACTCheck v0.84.29 · Regimen-level drafts; prescriber and oncology-pharmacy verification required